

Discharge Summary

Dear Colleagues,

Mr. Günther Vogt, DOB 07/25/1953, was admitted on 05/12/2025 with a 2-week history of a neuroischemic ulcer on the plantar surface of the right first metatarsal head, with surrounding cellulitis and systemic infection signs (fever 38.4°C).

Primary Diagnosis: Type 2 diabetes mellitus with diabetic foot syndrome (E11.6)

Laboratory values on admission:

Parameter	Value	Reference
HbA1c	10.8%	< 7.5%
CRP	42 mg/dL	< 0.5 mg/dL
WBC	14.2 G/L	4.0–10.0 G/L

Past Medical History: Type 2 diabetes for 18 years, on insulin glargine 28 units and metformin 500 mg twice daily. Peripheral arterial disease, ABI right 0.62 (measured 2 years prior). Stage 3b CKD, eGFR 38 mL/min — limits antibiotic and analgesic choices. Moderate diabetic peripheral neuropathy on nerve conduction studies 18 months ago. Hypertension on amlodipine 10 mg and lisinopril 10 mg. Ex-smoker, 30 pack-years, cessation 2010. Lives alone. Two prior left-sided foot ulcers managed conservatively.

Vascular Assessment: Urgent vascular surgery consultation day 1. Duplex sonography: severe stenosis of the right superficial femoral artery at the adductor canal with reconstitution below the knee. PTA recommended and scheduled as outpatient in 3 weeks.

Hospital Course: IV piperacillin-tazobactam initiated on admission. Sharp debridement day 2 under local anaesthesia — necrotic tissue removed, clean wound base approximately 3 × 2 cm exposed. Wound swab: polymicrobial flora including MRSA and Pseudomonas. Antibiotics adjusted to IV vancomycin (renally

dosed) and IV ceftazidime on day 4 per sensitivities. Total contact cast applied day 4 for offloading.

Glucose management intensified. Short-acting insulin aspart 6 units with each meal added to the glargine basal regimen on day 2. Fasting glucose improved from 280 mg/dL on admission to 138 mg/dL at discharge. Podiatry wound reviews days 3, 5, and 7 — progressive reduction in erythema, early epithelialisation at margins.

Multidisciplinary Review: Diabetic foot care team — diabetologist, vascular surgeon, podiatrist, and specialist nurse — conducted a joint review on day 5. Conservative surgical approach confirmed; total contact cast maintained. Plans for vascular angioplasty confirmed. Podiatrist recommended custom insole provision following wound healing to reduce recurrence risk.

Discharge Planning: Given that Mr. Vogt lives alone, detailed discharge planning was initiated from day 3. District nursing arranged for daily wound dressing and insulin support. His daughter was contacted and agreed to check on him daily for the first post-discharge week. A key safe already in place at the flat.

Medication at Discharge: Insulin glargine 30 units (increased), insulin aspart 6 units with each meal (new), metformin 500 mg twice daily, amlodipine 10 mg, lisinopril 10 mg, oral co-trimoxazole DS 14-day course, omeprazole 20 mg.

Follow-up: Podiatry wound clinic in 3 days. Vascular surgery for SFA angioplasty in 3 weeks. Diabetology in 2 weeks. District nursing daily.

Antibiotic Stewardship and Microbiology Review:

The antimicrobial stewardship team reviewed Mr. Vogt's antibiotic therapy on day 5. The wound swab had grown polymicrobial flora including MRSA and *Pseudomonas aeruginosa*. Vancomycin therapeutic drug monitoring was performed — initial trough levels were below target and the dose was adjusted to achieve AUC/MIC-guided dosing within the constraints of impaired renal function. Ceftazidime was selected over piperacillin-tazobactam for superior anti-pseudomonal activity based on the sensitivity results. The stewardship team recommended a total antibiotic course of 14 days from the date of adequate surgical debridement, with transition to oral co-trimoxazole as the final sensitivities confirmed susceptibility.

Community Nursing and Social Support Arrangements:

Given that Mr. Vogt lives alone and the complexity of his discharge requirements, a discharge planning meeting was held on day 6 with the patient, the diabetic foot nurse specialist, and the hospital social worker. The following arrangements were confirmed: district nursing to visit twice daily for the first week post-discharge for wound dressing and insulin administration review, then daily thereafter; a meal delivery service was arranged as the patient would have limited mobility in the total contact cast; his daughter agreed to visit daily for the first two weeks; and a telehealth check-in with the diabetic foot nurse specialist was scheduled for day 3 post-discharge.

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